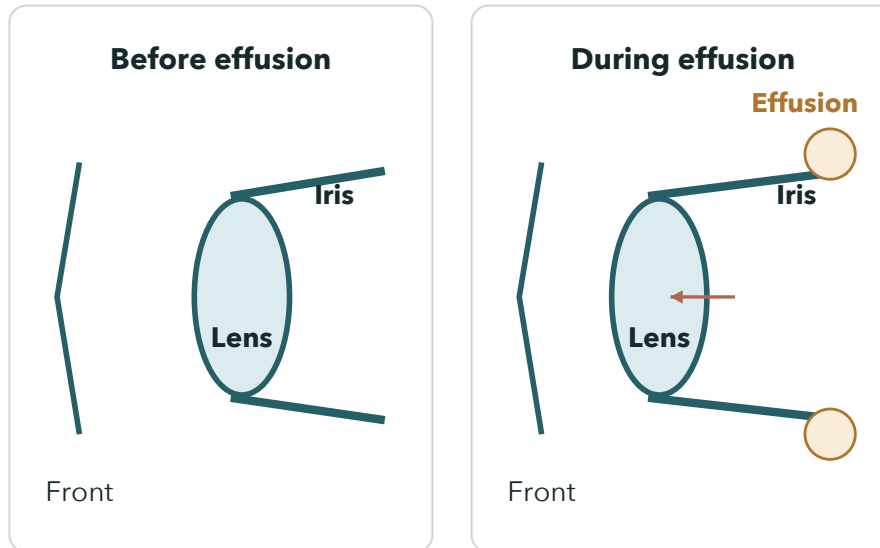


Angle closure after the first intervention

Check whether the remaining mechanism is pupil block, effusion or plateau iris.

Bilateral myopic shift points behind the iris

A | Effusion moves the lens-iris diaphragm



Bilateral myopic shift + medication change
Remove trigger; reverse the effusion mechanism

Original section comparing position before and during effusion. Recent topiramate exposure is a key medication clue.

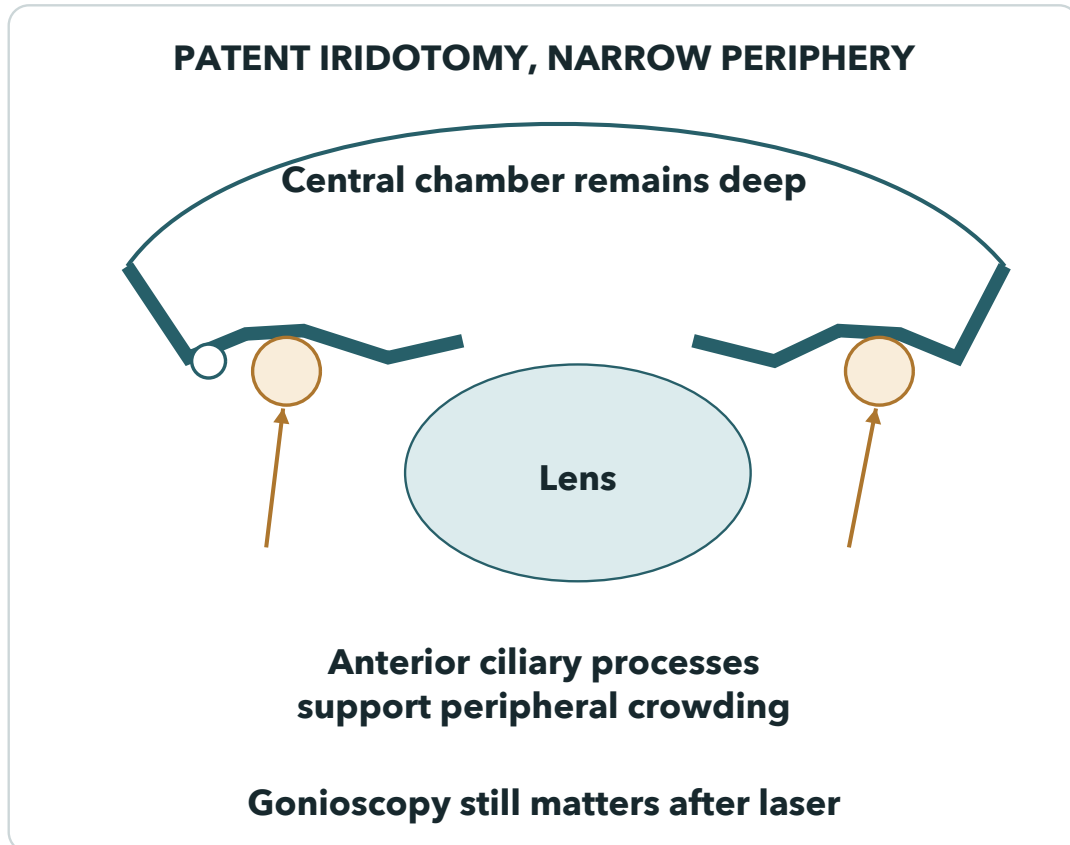
Mechanism changes treatment

Mechanism	Principle
Primary pupil block	Pressure reduction and an iridotomy bypass address the block.
Drug-related effusion	Coordinate trigger withdrawal; specialist cycloplegia, anti-inflammatory and pressure treatment.
Pilocarpine in effusion	May worsen the forward shift; an iridotomy does not reverse this cause.
Both eyes affected	Ask about medication changes and new refraction, not only familiar narrow-angle triggers.

References AAO EyeWiki | Primary and secondary angle closure | RCOphth | Angle closure guideline, 2022

A patent hole does not guarantee an open angle

B | Residual peripheral crowding



Original plateau-iris section: central chamber may be deep while ciliary processes support the peripheral iris against the angle.

Review both eyes

Persistent contact after patent iridotomy needs gonioscopy and reassessment for plateau iris, synechiae, lens effects or other secondary mechanisms. A double-hump contour on indentation supports plateau iris; further treatment is specialist and mechanism dependent. After a primary pupillary-block attack, an occludable fellow eye warrants prompt preventive iridotomy rather than waiting for symptoms. This high-risk fellow-eye setting differs from blanket laser treatment of every low-risk asymptomatic suspect found by screening. The asymptomatic fellow eye after a confirmed primary block attack can need prevention despite normal acuity and pressure. A patient with bilateral new myopia after topiramate needs a different emergency plan because the iris and lens have moved forward together.